Nivolumab and ipilimumab for pleural mesothelioma

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Every 42-day cycle · Day 1 nivolumab + ipilimumab

Active medicines: nivolumab + ipilimumab

Emetogenic risk: low

Medicine	Dose / route	When
Ondansetron	8 mg orally	Day 1 before treatment, if following the NCIS low-risk default Individualise/local policy. No automatic antiemetic corticosteroid course.
Metoclopramide	10 mg orally three times daily as needed	Subsequent days as prescribed Specify duration and check contraindications/interactions.

Additional phase-specific support

- Both immune medicines can inflame healthy organs during treatment or after it stops. Keep your immune alert card and report new symptoms promptly, including for at least five months after the last dose.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00792 v2 · pNCCP 00792 p5; NCIS V8 Table 1 p4 · NCIS low-risk row

Sources checked 2026-10-29 · SACTCheck v0.84.27 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Every 42-day cycle · Day 22 nivolumab alone

Active medicines: nivolumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Both immune medicines can inflame healthy organs during treatment or after it stops. Keep your immune alert card and report new symptoms promptly, including for at least five months after the last dose.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00792 v2 · pNCCP 00792 p5; NCIS V8 Table 1 p4 · Current NCCP regimen

Sources checked 2026-10-29 · SACTCheck v0.84.27 · Regimen-level drafts; prescriber and oncology-pharmacy verification required